

This study reworks the concept of genre from rhetorical, dialectical, and dialogic perspectives. From these perspectives, genre is redefined as a stabilized-for-now or stabilized-enough site of social and ideological action. This definition is then applied to a specific literacy practice—medical record keeping—evolving in a specific context—a veterinary college. Data were gathered during a 6-month ethnographic study of the college. The larger research project focused on the teaching and learning practices that constituted literacy, i.e., the ways of speaking, reading, writing, and listening characteristics of veterinary medicine. The project consisted of interviews, observation, and document collection. Triangulation was achieved both within and between methods. Data were analyzed using Glaser and Strauss's "grounded theory" techniques. When the concept of genre is applied to medical record keeping, the complexity of this literacy practice becomes apparent. A specific record-keeping system—the Problem Oriented Veterinary Medical Record (POVMR) system—was the site of intense controversy at the college. The system articulated a set of values that one group of faculty and clinicians espoused and another group rejected. The system itself was embedded in the exam structure of the college, and a good deal of evidence emerged that the POVMR itself was promoting certain types of literacy abilities and making others less likely.

Records as Genre

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Researcher: Do you think there is a problem with literacy at the college?

Dr. L: Yes.

Researcher: How do you see it?

Dr. L: It's an unwillingness or inability to write in English. They have no difficulty in spoken English.

Researcher: Do you see this in essays or in exams?

Dr. L: I'm appalled by the techniques students use in answering questions on exams. I don't know if they are unable to write on exams or if it's the way they have been trained. If you write a logical question—asking them to describe briefly or outline—you get back a string of words. There are no connections—just arrows pointing from one thing to

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another. Isolated words are supposed to speak for themselves. As an examiner, I'm supposed to find the isolated words and put them into some kind of order.

Researcher: What about in your surgery labs, do they have any writing attached to them?

Dr. L: They get training in medical records, and they are graded for their records. But they are graded more for their observations than for their literacy skills.¹

This exchange occurred during an ethnographic study of literacy at a veterinary college (Schryer, 1989), where members of the curriculum committee had called me in as a consultant. They wanted to know if there was a problem with literacy among their students. In response, I asked them three questions: (a) What did literacy mean in their professional context? (b) Were students being taught these characteristic ways of speaking, writing, reading, and listening? and (c) Was this literacy appropriate or sufficient for students' future professional needs? In other words, rather than simply examining student texts in order to taxonomize errors (and thus blame students for any inadequacies), I wanted the opportunity to ask the radical, contextual question—what did literacy mean here—and the opportunity to observe the teaching and learning practices² that constituted literacy at the college.

The project, an ethnographic study (reflecting the work of Shirley Brice Heath³ [1983],⁴ David M. Fetterman [1984], and literacy researchers such as Kenneth Levine [1986] and Brian Street [1984]), was conducted between January and July of 1988. It consisted of 80 interviews with students, faculty, and practitioners; over 200 hours of participant observation in classrooms, labs, and clinics; 10 reader protocols of faculty evaluating student papers;⁵ extensive document collection including exams, minutes of meetings, student papers, student applications to the college, and course evaluations; and a review of medical and veterinary literature on education.⁶

Experienced ethnographers (Galton & Delamont, 1985; Goetz & LeCompte, 1984) have identified triangulation as a central concern for qualitative researchers. It is through triangulation, or detailed cross-checking and cross-referencing, that valid generalizations emerge from ethnographic research. Two types of triangulation were used in this study: between-method triangulation and within-method triangulation. In other words, not only did I use a variety of methods (interviews, observation, document collection) but I also searched for

varied and balanced sources within a specific method. For example, I interviewed selected faculty in all disciplines at the college and asked all participants the same set of questions as well as particular questions reflecting their own interests and expertise.⁷ Data were compiled in field notes constructed in fairly traditional ethnographic style. I tried to observe the distinction between observational and analytical notes. Observational notes contained detailed descriptions of what I observed and detailed reconstructions of conversations that used as much of the "original" language of participants as possible. Analytical notes reflected my more personal, more analytical commentary.⁸ These field notes were then analyzed using "grounded theory" techniques (Glaser & Strauss, 1967). I located a range of categories that seemed relevant and then moved the data (specific comments and documents, for example) into appropriate categories. It was during this process that I became aware of the importance of medical record keeping. A number of comments and observations began to appear under this category as did cross-references between medical record keeping and evaluation practices. It is perhaps due to these research techniques that the analysis presented in this article appears as a series of generalizations supported by data derived from an array of sources rather than as an ethnographic narrative.

Traditional ethnographic research often includes a map in order to help locate readers. The following metaphorical map not only outlines the physical dimensions of the college but also tries to convey some of the complex connections between various subgroups and their teaching and learning practices. At the time of the study, this medium-sized college in eastern North America followed a traditional curriculum, a curriculum reflected in the very structure of the college. Students and visitors enter the college through an imposing 19th-century entrance way. Attached to this building is the Department of Biomedical Science. As is traditional in many medical schools, students take courses such as anatomy, physiology, or biochemistry from members of this department. Biomedical faculty, for the most part, believe that students must have an extensive background in scientific facts prior to their involvement in clinical problem solving. Many believe that clinical cases are too complex, too multifactorial, and that students are too prone to hasty diagnoses without a great deal of prior course work.

After walking through this oldest section of the complex, the visitor descends the stairs and walks through a series of tunnels to arrive at the Clinical Studies area and beyond that to the large and small animal

hospitals extended on either side of the college. This joint hospital area is the heart of the college—especially for students.

Two departments, Clinical Studies and Population Medicine, occupy this part of the college. Clinical Studies consists, for the most part, of clinicians—specialists with Board accreditation rather than doctorates—who work in the animal hospital and supervise and teach students in the last 2 clinical years of the program. This group represents a wide range of interests. The primary division involves specialization in either large or small animals, a division that also reflects rural/urban and, to a lesser extent, male/female differences. Large animal practitioners tend, for example, to be males from a rural background. As a group, clinicians are concerned about students' ability to solve medical problems and provide adequate care for the animals under their supervision.

Whereas faculty in Clinical Studies concern themselves with the diagnosis and treatment of individual animals, instructors in Population Medicine focus on the prevention and treatment of disease among large groups of animals. Population Medicine, the newest department in the college, has been charged with bringing consultancy work into the college as well as with developing a new focus on epidemiology. Faculty in this department tend to stress problem solving and research skills.

As the visitor proceeds down through the corridor, past the hospital, he or she arrives in the newer area of the college and the last two departments—Virology, Microbiology, and Immunology (VMI) and Pathology. VMI instructors share similar interests and beliefs with faculty in Biomedical Science. They, too, are researchers as well as teachers. They have a specific set of concerns, however. Because career streaming was not an option for veterinary students, these faculty felt forced to compress vast amounts of information into their courses. For example, veterinary students took one microbiology course in which their instructor tried to provide them with information regarding all the microbes that could affect all species except humankind.

Finally, Pathology, like Population Medicine, shares an interest in both research and clinical practice. Pathologists have close connections to the clinics because the department has its own final-year rotation (a 1- or 2-week period students spend working with pathologists) and because pathologists perform the diagnostic and postmortem clinicians need to confirm or rule out diagnoses. For this reason, pathologists understand the problem-solving strategies used by clini-

cians. At the same time, pathologists view themselves as researchers and thus have a strong interest in developing scientific knowledge.

Dr. L, whose comments began the article, was a faculty member and senior veterinary surgeon specializing in large animals. During my research, I heard his complaint echoed again and again. Graduating students, my informant kept telling me, were susceptible to the "Green Grad" syndrome. This syndrome, according to informants, consisted of staccato oral delivery and a disjointed writing style that often seemed oblivious to particular audiences.

The exchange, however, between Dr. L and me also reveals a fundamental contradiction. Dr. L viewed literacy in terms of writing exams but did not see that the keeping of medical records was also a form of literacy. Further, neither he nor other members of the college were aware of how much record keeping the students actually did or of how the record-keeping system might be influencing the faculty's teaching and evaluation practices and thus students' learning and literacy. Short-answer exams, for example, were the dominant form of testing at the college. The most prevalent type of question on these exams, especially in the final 2 years of the program, incorporated the problem-solving structure of the record-keeping system (see exam question discussed later in this article). Consequently, my research began to focus on the effect of the genre of record keeping on members of this discourse community.⁹

Two sets of questions emerged from the study. First, why examine record keeping at all? Second, is the concept of "genre" an accurate or useful way to theorize about these texts? Is genre a useful way to talk about the ways of speaking and writing characteristic of discourse communities (i.e., their contextual literacy)?

This article argues that record keeping is a central discursive practice in disciplines such as psychiatry, social work, and medicine. These powerful communities affect all of us, and their record-keeping practices shape much of the socialization within these communities and the subsequent interaction of the communities with the rest of society. Second, this article maintains that the concept of genre, when viewed from rhetorical, dialectical, and dialogic perspectives, can illuminate much of the work and ideology of such textual practices. From these perspectives, genres can be described as stabilized-for-now or stabilized-enough sites of social and ideological action.

Researchers such as Catherine Pettinari (1988) and Lucille McCarthy (1991, 1992) have suggested that activities related to record keeping are central discursive practices.¹⁰ Pettinari pointed out that medical

records, besides providing a means of communication and planning, actually come to represent patients themselves. R. Frankel (1981) noted, for example, that in videotapes of doctor and patient interactions, physicians were observed consulting records before questioning patients. McCarthy (1991) explored how a "charter document," the *Diagnostic and Statistical Manual of Mental Disorders (DSM-III)* affected the diagnostic and treatment practices of psychiatrists. Her most recent work (1992) on that profession's revision of the manual suggested the ways that the charter document itself continues to shape the mental health professions. For example, the acknowledgment of the existence of a syndrome means that a specific set of symptoms became a "real" medical condition for practitioners. Thus Bulimia nervosa was a recognized eating disorder in the most recent revision of the manual, whereas Binge Eating Disorder (BED) was still under dispute as a recognized disorder.¹¹

The sociologist Dorothy E. Smith (1974, 1984, 1987) observed that the record-making practices of social and medical agencies constitute the "documentary reality" (1974, p. 257) that constrains and constructs our social and physical existence. Our lives, she said, are "infused with a process of inscription" (1984, p. 59) or the rendering of an event or object in documentary form (p. 65). Documents such as records are the very substance of organizations, their fact-making mechanisms. They are "the forms that externalize social consciousness in social practices, objectify reasoning, knowledge, memory, decision-making, judgement, evaluation . . ." (p. 60). Neither sociologists nor researchers into discursive practices can afford to ignore these deeply shared ways of rendering phenomena into language. Speaking of records, Smith (1987) observed:

Much of the phenomenal universe of sociology has its ground in these institutional processes which organize . . . and control contemporary society. They provide a work of ruling. Phenomena providing much of the familiar stuff of sociology, such as mental illness, violence . . . come into being as integral constituents and products of the bureaucratic, legal and professional operations of this apparatus. They exist as properties of the relations in and through which society is ruled. Their objectified forms are not expressions of the actualities of a naturally existing world but are the artful constructions of text-based methodologies and the practices of formal organization. They constitute a world known in common through the medium of texts and vested in organizational systems of record keeping. (p. 151)

Smith suggested, too, that these textual practices remove from the individuals who use them much freedom of action and expression. From Smith's perspective, records not only constitute organizations but are also constituted by them and function as mechanisms of control. In fact, much of Smith's research explores the ways such textual practices often systematically exclude women.

As researchers into professional discourse, we need to account for the textual practices related to record keeping. As the case study that follows illustrates, the genre of record keeping has a host of textual, cognitive, and social consequences both for the discourse community that uses it and for those who interact with or are influenced by that community.

"Terms," Thomas McLaughlin (1990) reminded us, "commit us to particular values, and if we are aware of these commitments, we can *take* the position we inhabit" (p. 7). Certainly we need to examine critically the consequences of labeling record keeping as a genre. Genre, of course, has a long history as a literary taxonomic term. Laurent Jenny (1982), for example, noted that "we grasp the meaning and structure of a literary work only through its relation to archetype . . ." (p. 34). Although rhetoricians such as Carolyn Miller (1984) rejected the rigid classifying impulse of literary genres, Miller acknowledged that a theory of genre suggested that resemblances could exist between texts at the levels of substance, form, and style. Quoting Karlyn Kohrs Campbell and Kathleen Jamieson (1978), Miller (1984) initially defined genre as not merely consisting of "a series of acts in which certain rhetorical forms occur. . . . Instead a genre is composed of a constellation of recognizable forms bound together by an internal dynamic" (p. 21). This dynamic, Miller insisted, "fuses substantive, stylistic, and situational characteristics" (p. 152). Researchers such as Katherine Rentz (1989) have recognized the cognitive implications of Miller's definition. In her attempt to revitalize genres as a useful theory, Rentz (1989) suggested that genres were not simple, rigid formulas but rather "recurring verbal responses to particular shared contexts; . . . verbal schema, acquired over the course of repeated readings, stored in the mind as interpretive tools, and used or revised as new phenomena come in" (p. 197). Her definition recognized that genres both shaped reading and writing practices and were shaped by the texts in which they were embedded.

However, both Miller and Rentz were less interested in the textual or cognitive dimensions of genre than they were in the possibility that genres were a contextual phenomenon. As Miller (1984) noted in her

introduction, she wanted "to make of rhetorical genre a stable classifying concept" and at the same time ensure that "the concept is rhetorically sound" (p. 151) or responsive to both audience and exigency. Thus Miller redefined genre as "a recurrent, significant action" (p. 165). Each of these three terms—*recurrent*, *significant*, and *action*—reflects key insights into the nature of genre. As a recurrent phenomenon, a genre represents a series of texts sharing features at the levels of content, form, and style. At the same time, this notion of recurrence involves the social actors who construct the genre. Only those involved can interpret a situation and any possible responses as recurrent and significant. Social actors involved in a particular context recognize a text as either aberrant or important and as sharing features with related texts. For this reason, a genre is an *emic*¹² phenomenon.

To understand why a typical response has developed or how it works, researchers must consult the interpreters of both the genre and the situation. In Miller's terms, we need to conduct "ethnomethodological" research and seek "to explicate the knowledge that practice creates" (p. 155). From Charles Bazerman's (1988) perspective, Miller was revealing the "decorum" (p. 320) at work within typical responses to recurrent situations. Only those who constantly construct their social worlds using these "forms of life embodied in current symbolic practice" (Bazerman, p. 320) can illuminate for outsiders their ways of speaking and writing and explain their significance.

Most important, both Miller and Bazerman agreed that rhetorical genres were a form of social action. A genre coordinates work from the simplest action of constructing a shopping list (Witte, 1992) to the complex activity involved in conducting scientific research (Bazerman, 1988). Thus a genre is a frequently traveled path or way of getting symbolic action done either by an individual social actor or group of actors.

Miller's redefinition of genre goes a long way in helping us theorize about perceived recurrent ways of using discourse. She asked us to examine textual products for shared formal characteristics; she reminded us that recurrent forms have cognitive consequences for their users; she made the concept of genre rhetorical in that she connected it to both audience and exigency; finally, she asked us to consider the actual work or social action that a genre coordinates. She elaborated a useful theory in that it asked us to consider a possible genre from textual, cognitive, and contextual or rhetorical perspectives.

However, as Miller appears to admit, her original purpose in "Genre as Social Action" was somewhat contradictory. She wanted

"to make of rhetorical genre a stable classifying concept" and at the same time ensure that "the concept is rhetorically sound" (p. 151). But if genre is used to describe stable systems, then the concept is rhetorically unsound because a stable system cannot respond to changes in audience or circumstance. Miller realized that if genre were conceived as conventional "ways of acting together" (p. 163), then the concept did not lend itself to taxonomy "for genres change, evolve, and decay" (p. 163). Miller left this essential contradiction unresolved. In fact, much of the current work on genre attempts to locate the stabilizing elements in language and cannot explain the evolving and contextual nature of genre itself. Charles Bazerman (1988), in his extensive exploration of the genre of the scientific experimental article, defined genre as "a sociopsychological category which we use to recognize and construct typified actions within typified situations. It is a way of creating order in the ever-fluid symbolic world" (p. 319). John Swales (1986) saw genre as a "recognized communicative event with a shared public purpose . . . and within variable degrees of freedom, a structured and standardized communicative event with constraints on allowable contributions in terms of their positioning, form, and intent" (p. 4). Yet as Stephen Witte (1992) has noted in his observation of complex communication events, genres appear to vary both externally and internally according to community needs and rhetorical circumstances. Genres vary externally when social actors have a choice of genres with which to fulfill a task. They vary internally when social actors working within a genre have a choice of strategies or forms. Anyone who has observed reports in various organizations knows that what "counts" as a valid report changes from organization to organization in terms not only of content but also of form and style.

When genres are viewed from a rhetorical as well as a dialectical and dialogic perspective, they then become ways to theorize about complex, evolving discourse practices. Thus genres can be described as stabilized-for-now or stabilized-enough sites of social and ideological action. This definition attempts to build in a sense of a particular genre as a simultaneously diachronic and synchronic structure. All genres have a complex set of relations with past texts and with other present texts: Genres come from somewhere and are transforming into something else. They are heavily conventionalized and yet contain inherent contradictions so that their users have internal options and thus some freedom of expression, depending on the genre. Genres, because they exist before their users, shape their operators; yet their users and their discourse communities constantly remake

and reshape them. Genres are inherently ideological; they embody the unexamined or tacit way of performing some social action. Hence they can represent the ways dominant elite do things. In other words, to echo Bakhtin (1981),¹³ genres are sites for the centrifugal and centripetal forces that struggle to maintain and yet renew discourse practices.

Such a view of genre reflects developments in dialectical anthropology and, of course, the insights of Bakhtin. Robert F. Murphy, in *The Dialectics of Social Life* (1971), provided an overview and synthesis of what he called "dialectical reasoning" (p. 85). This perspective, he suggested, was rooted in the Sophists. Their thought acknowledged the involvement of the observer in the observed and thus the fragmentary, constructed nature of all knowledge. They situated all events in time, in a constant process of coming-to-be and viewed discourse as inherently contradictory. G. B. Kerferd (1981) explained this attitude to language and experience as follows:

But the world of experience is characterised by the fact that all or most things in it both are and are not. Therefore language must also exhibit this structure. This it must do by giving expression to two opposed logoi concerning everything. (p. 72)

Murphy suggested that dialectical reasoning has had a rich intellectual history, informing the thought of Freud, Marx, and Hegel among many others. He provided a detailed description of the operations of this intellectual tradition. "The principal characteristic of a dialectic," he suggested, "is that it is critical and sceptical of received truth . . ." (Murphy, 1971, p. 87). Such a critical attitude was necessary because "dialectical thought . . . denies the objectivity of the here and now through the assertion that both disappear, or negate themselves, upon being considered in time" (p. 94). This diachronic perspective infuses dialectical thought with creative negativity or transformativity. Murphy (1971) suggested that

things are not to be understood as fixed entities but are in a continual state of transformation into other forms of themselves. . . . States of being contain their contradictions as a condition of their existences, and they realize their possibilities by transcending these limitations and passing on to another phase. But the latter is also found to breed its own opposition, and the resultant world view is one of continual movement and transformation. (pp. 94-95)

Anthropologists using this approach assume that complex dialectical relations exist between individuals and their social groups, between language and thought, between normative beliefs and behavior. Each side of these binary oppositions affects the other but never in a totalizing or isomorphic fashion. Contradictions always exist. It is through contradictions, in fact, that change occurs. This sense of complexity and creative contradiction is reflected in the term *transformativity*. Genres are, in fact, characterized by transformativity.

Language and other symbolization processes, Murphy asserted, attempt to objectify and fix the communication process. Yet the attempt is futile. As both the Sophists and more recent dialectical thinkers such as Raymond Williams (1980) and Andre Giroux (1985) recognized, discourse (and consequently genre) itself is a site of ideological or hegemonic action and thus of contradiction and consequent transformation. Hegemony, for Williams, expresses the domination of a particular class. Yet hegemony is not just a mere set of beliefs or values. He asserted that

hegemony supposes the existence of something which is truly total, which is not merely secondary or superstructural . . . but which is lived at such a depth, which saturates the society to such an extent, and which . . . even constitutes the substance and limit of common sense for most people under its sway, that it corresponds to the reality of social experience. (Williams, 1980, p. 37)

Although powerful, hegemony, according to Williams, is not unitary. He emphasized that "hegemony is not singular; indeed . . . its own internal structures are highly complex, and have continually to be renewed, recreated and defended; and by the same token, that they can be continually challenged and in certain respects modified" (p. 38). William's theoretical model recognized the ideological forces of stability and yet the existence of other contradictory and equally ideological forces of transformation. Giroux (1985) in his introduction to Paulo Freire's *The Politics of Education*, worked from a similar perspective, noting that Freire's social critique recognized that "the logic of domination represents a combination of historical and contemporary ideological and material practices that are never completely successful" because they "always embody contradictions" (p. xii). From this perspective, every genre is ideological. A set of "commonsense" values and practices is embedded within the accepted way of doing

things. Yet contradictions always exist, so either some range of choice is possible or the genre itself can be critically analyzed.

M. M. Bakhtin reflected on the nature of genre from his earliest to his latest work. In the earlier "Discourse in the Novel" (1981), Bakhtin identified genre as one of the centripetal or homogenizing, hierarchical forces within language. "Stratification," he suggested, "is accomplished first of all by the specific organisms called *genres*" (p. 288). Like Miller, Bakhtin saw a genre as a fusion of content and style. He suggested that "certain features of language (lexicological, semantic, syntactic) will knit together with the intentional aim and with the overall accentual system inherent in one or another genre: oratorical, publicistic, newspapers . . ." (p. 288). As Bakhtin made clear, these discourse structures were also imbued with "concrete value judgments" (p. 289). He suggested that "they knit together with specific objects and with the belief systems of certain genres of expression and points of view peculiar to particular professions" (p. 289). Bakhtin's earlier work added to genre theory research questions related to ideology. But several questions related to the specific unit of analysis and the actual ways genre might work to stabilize discourse remain unanswered.

The actual unit of analysis described by the term *genre* is difficult to determine in complex communication situations. For example, is an academic conference a genre? Decorum, to echo Bazerman, rules at such events. Specific content and language practices are required for successful linguistic performances at conferences. Yet delegates can deliver papers, organize workshops, or participate in discussions—all literacy events that require differing but overlapping language practices. Also, when participants are involved in such complex events, how do they know which set of practices to initiate at any one time?

In his later work, especially "The Problem of Speech Genres," Bakhtin (1986) provided his most thorough exploration of issues related to the stability as well as questions related to the transformativity or instability of genres. Bakhtin's concept of genre was intricately linked to his dialogic understanding of language and his refusal to accept the Saussurean division between *langue* and *parole* and the consequent attempt to create a rule-governed science of language. "Speech genres," as Bakhtin referred to genre, or "concrete utterances" (p. 63), not syntax, are the essential linguistic units. These fusions of content, style, and compositional structures, inseparably

linked to specific contexts, are the ways language enters life. The "utterance" is the "*real unit of speech communication . . .*" (p. 71). The boundaries of genre or concrete utterances are determined by a "*change of speaking subjects . . .*" (p. 71, emphases in original). In an important development of this insight, he continued:

Any utterance—from a short (single word) rejoinder in everyday dialogue to the larger novel or scientific treatise, has, so to speak, an absolute beginning and an absolute end: its beginning is preceded by the utterances of others, and its end is followed by the responsive utterances of others. . . . (p. 71)

Concrete utterances exist as either primary (simple) or secondary (complex) speech genres. Secondary speech genres such as "novels, dramas, all kinds of scientific research . . . arise in more complex and . . . organized cultural communication . . ." (p. 62). These more complex genres absorb the simpler ones. This operation is at work, for example, in complex poetic texts such as William Carlos Williams's *Paterson*, a long poem that incorporates personal letters, sections of medical texts, dialogue, and lyric passages. This process also seems to be at work in medical records that can encompass operating reports (Pettinari) or consultants' reports.

Bakhtin's most important insight, however, is that the unit of analysis remains the same—whether the genre is a one-word rejoinder or a scientific article. The change in speaking subjects determines the boundaries of the genre. This notion has transformative implications. A concrete utterance is a response, or a link in a dialogic chain. It is part of an ongoing conversation. As part of that conversation it carries echoes and memories of previous conversations. A concrete utterance is coming from somewhere. At the same time, genre is going somewhere. It is a rejoinder requiring another response. Those involved know when the genre has been enacted or finalized because the possibility of response opens up.¹⁴ When the command is given, when a draft or final version of a journal article is finished, when the record is complete, a response becomes possible. The command might be followed, the article rejected, the record used as an audit of the practitioner's activities.

The creative tension that exists in Bakhtin's notion of genre, its balance between stability and transformativity, is caught in this notion of the concrete utterance bounded by a change of speaking subjects. Subjects choose particular speech genres based on their linguistic

experience, and all linguistic experience occurs through the utterance. Built into these utterances is a sense of "addressivity" or attitude to the "other," the audience. The audience shares these utterances and sense of addressivity, or they could not know when to respond so the dialogue would cease. This aspect of genre constitutes the essential and relatively stable, normative side of genre. Yet Bakhtin also suggested that speech genres are "changeable, flexible and plastic" (p. 80).

External and internal reasons exist for this centrifugal or destabilizing, transformative action within genres. Externally, any genre is part of a vast heterogeneity of other genres from the most stable and monologic (often associated with high social status) to the more unstable and dialogic (often the more familiar). Any speech community has a rich repertoire of such genres in a constant dialectic with each other and thus constantly influencing and changing one other. Internally, any utterance is also filled with "*dialogic overtones*" (p. 92, emphasis in original). It is a response preparing for a rejoinder, thus containing words forged in other contexts, other utterances. Bakhtin observed that no matter how monologic the utterance (even a scientific or philosophical treatise), "it cannot but be . . . a response to what has already been said about the given topic, on the given issue. . . . It [responsiveness] will be manifested in the overtones of the style, in the finest nuances of the composition" (p. 92). "After all," Bakhtin asserted, "our thought itself—philosophical, scientific, and artistic—is born and shaped in the process of interaction and struggle with others' thought, and this cannot but be reflected in the forms that verbally express our thought as well" (p. 92). These echoes, rejoinders of other texts and other genres, often contain "foreign" or contradictory elements.

From Bakhtin's perspective, even the most monologic and thus ideological genres such as scientific articles or medical records are sites of struggle between the centripetal and centrifugal forces within language. As researchers, we can explore genres in the productive ways suggested by Miller, Bazerman, Murphy, and Bakhtin. We can locate our research in the context of the speech communities that use these apparently recurrent ways of using language. Thus we can look for stabilizing elements especially in the most monologic and thus most inherently ideological genres. But we can also look for sites of contradiction and thus transformativity. We can ask questions about the work a genre does and does not do. We can ask who has access to a certain genre and who does not.

MEDICAL RECORDS AS A GENRE

The implications of this expanded notion of genre become clearer when the rhetorical insights of Miller and Bazerman, the dialectical impulse of Murphy and Williams, and the dialogic perspective of Bakhtin are applied to a specific genre—record keeping—at a specific location, a veterinary college.

Medical Records—History

The medical record system adopted at the college is the Problem Oriented Veterinary Medical Record (POVMR) system.¹⁵ A variant of a human medical record system called the Problem Oriented Medical Record (POMR) system, the POVMR reflects a particular position in the current struggle within the veterinary medical establishment to reform both the nature of medical education and the professionalism of medical practice.

The POMR system was developed in the 1960s for human medicine by Dr. Lawrence Weed (1969) for two reasons. Weed, like other commentators (Schon, 1983, 1987), was concerned about the effect of traditional medical education, which encouraged students to memorize a vast number of facts without emphasizing the problem-solving skills essential for the practitioner. For this reason, he wanted a record-keeping system that would mirror the clinical problem-solving strategies used by the best practitioners. At the same time, Weed wanted to make medical records more readable, more scientific—in effect, more open to monitoring, evaluation, and standardization than traditional records.

The significance of the changes Weed proposed becomes clearer when we understand the system he was critiquing. Prior to Weed, record keeping had been generally haphazard but basically dominated by the Source-Oriented Record (SOR) system. Under the SOR system, the practitioner recorded the patient's complaints, a review of symptoms, and physical exam results. This initial information was followed by "a statement of the most likely provisional diagnosis, often accompanied by one or more differential diagnoses" (Tugwell & Dok, 1985, p. 148). Below the diagnosis, the investigation and management of the case were listed together. Progress notes followed the same format with no standardized approach for recording. Several kinds of information—test results, observations, diagnoses—were

recorded sequentially. In fact, the SOR followed a fairly narrative structure.

Weed was intensely critical of the SOR system. He believed that the system encouraged physicians to jump to diagnostic conclusions and to ignore the complexity of problems that could afflict a patient. He felt that medical records, because of their failure to break problems into manageable units and their failure to connect those problems to subsequent investigation and treatment, were often comprehensible only to their initial writers. Thus medical records could not be used for either evaluation or research purposes and could be sources of mismanagement when more than one health care provider was involved in a case. Finally, Weed saw the possibility that records could be redesigned to imitate a specific medical problem-solving process. They could become in his words "audits of action" (introduction). From a rhetorical perspective, Weed's system offered new physicians a heuristic that reflected the problem-solving structures that he believed many experienced physicians employed.

Medical Records—At the College

Although many North American medical colleges have adopted the POMR during the past three decades, the POVMR is a recent innovation at the veterinary college that I studied. POVMR proponents are a group of young clinicians eager to reform veterinary education and to increase the possibility of doing their own research in the clinical areas, a growing necessity as the field becomes more academic and professional. Clinicians who used the POVMR as a teaching practice praised its ability to involve students in clinical problem solving. Both the record technicians and many clinicians also recognized that records using the POVMR were more useful for research purposes than more traditional recording systems. Problems can act as a type of search variable for compiling data across sets of records.

Yet some clinicians, biomedical faculty, and, as we shall see, students and practitioners opposed the system. Some older clinicians, trained in the SOR tradition, have either left the clinic for administrative positions or have refused in less overt ways to cooperate with the implementation of the new system. One of the chief proponents of the POVMR admitted that it had taken his group about 3 years to get the system in place and that it was still a case of "banging other faculty

-
- I. Establishment of a data base
 - History
 - Physical examination
 - Admission laboratory work
 - II. Formulation of all problems
 - Problem List
 - III. Plans for each problem
 - Collection of further data
 - Treatment
 - Education of patient
 - IV. Follow-up on each problem
 - Progress notes: titled and numbered (Weed, p. 14)
-

Figure 1: Components of the POMR (Problem Oriented Medical Record)

over the head" to get them to accept it. Some biomedical faculty rejected the system because it implied teaching through cases. They believed that students needed "to possess scientific facts before they were overwhelmed with complex cases." One biomedical faculty member observed:

The POVMR is a good problem-solving approach and it works well in the clinics. But students need the language of veterinary medicine first and a basic understanding of the normal. The instructors of Pre-Veterinary and first-year courses (the basic science courses) frequently have heated discussions with second-year and up instructors (the more clinic-oriented faculty) because they forget that we have to provide students with the foundations.

Thus, as with all genres, the POVMR is linked to past texts and definitely instantiates a set of values for its proponents and detractors.

Records in Action

If records are a genre, then they facilitate social action, and thus it is important to examine the type of work performed by records both as a system and within the context of the college.

The attached illustration (see Figure 1) schematically outlines Weed's project.

The second illustration (see Figure 2) is a visual representation of the POVMR as it is presented to veterinary students in their Introduc-

-
- Data base
 - Signalment—age, sex, breed
 - Complaints—symptoms
 - History (hx)
 - Physical exam (pe)—signs
 - Problem list (all listed and numbered)
 - Diagnosis
 - Physiological finding
 - Physical finding
 - Abnormal lab finding
 - Initial plan
 - Diagnostic (dx)
 - Therapeutic (rx)
 - Client education (ce)
 - Progress notes (numbered according to problem)
 - Data
 - Lab results
 - Physical exam (pe) results
 - Flow charts
 - Consult notes
 - Assessment
 - Diagnostic rule outs
 - Explanations
 - Diagnosis
 - Prognosis
 - Plans
 - Diagnostic (dx)
 - Therapeutic (rx)
 - Client education (ce)
-

Figure 2: Components of the POVMR (Problem Oriented Veterinary Medical Record)

tion to Medicine course. The main, but not only, difference between the POMR and the POVMR lies in the structure of the progress notes. In human medicine, physicians record subjective data, gathered from the patient, and objective data, the physician's observations, as well as their assessment and plans (this process is called "soaping a case"). In veterinary medicine, subjective and objective data have been collapsed into a single category called data. This change has occurred because veterinarians cannot collect subjective data—animals cannot talk.¹⁶ Consequently, veterinarians speak of data, assessment, and plans or "daping a case" rather than "soaping a case."

Creating a Data Base

Completing each of the record's four steps has textual, cognitive, and ideological consequences. The clinicians at the college, like Weed, were adamant that good medical practice begins with a thorough and accurate data base. During their rounds in the final year of the program, students discovered the consequences of inadequate data collection—public humiliation and being sent back to the animal or owner to gather more information. Data collection in veterinary medicine begins with signalment, or information regarding the age, sex, and breed of the animal. In fact, veterinarians can predict a good deal about possible disease conditions based on this information alone. For example, some animals have been so inbred that many disease conditions are now hereditary. The signalment, then, cues practitioners to gather information about the particular animal—its specific history. Practitioners are taught to observe both symptoms and signs. According to course outlines and clinicians, symptoms are problems as viewed through the owner's eyes; symptoms are subjective data.¹⁷ Signs, on the other hand, are what the practitioner sees; they constitute objective information about the animal. Signs have greater epistemological status than do symptoms. Throughout their training, veterinarians, like other medical personnel, are often taught to abide by this distinction, a distinction that is encoded linguistically. For example, in the following excerpt taken from a round, a clinician (C) and a group of students were discussing a golden retriever that had been hit by a car. The student (S) responsible for caring for this case was responding to a series of rapid questions:

C: What do you look for?

S: Hidden fractures, pain.

C: You can't really miss fractures.

S: Ruptured bladders.

C: There's one other problem—traumatic myocardia. What are the signs?

S: Lethargy, weakness.

C: Those are the things the owner can see. What are the things you can see?

S: Pulse deficit, arrhythmias, myocardial infarction, aneurysm formation.

On several occasions, I observed both students and clinicians taking histories from clients. The technical language of signs, or "vet speech" as the students called it, sometimes intruded, and the data collectors had to rephrase their questions. In one instance, a student working on an animal's history with its owner, noted that the animal had been

"pyrexia" for a week. The worried owner immediately asked what that meant. The student explained that it simply meant "feverish." In fact, vet speech was often a source of amusement. During a pathology laboratory, two graduate students were role-playing a client interview for undergraduate students. The "veterinarian" mentioned that the animal had showed some signs of "polyuria." The "owner" quickly asked what that "Japanese" word meant.

Another quality of data collection not inherent in the record but present in practice was the rapid question and answer sequences that students were encouraged to use, as illustrated in an excerpt from a demonstration round. Demonstration rounds were informal, volunteer activities. During one noon hour a week, graduate students in the clinical area "demonstrated" cases for undergraduate students. About 50 students were present in a large room, graded so that students sat in wooden, bleacherlike seats above the central demonstration area. One door at the lower level led to the small animal clinic, and the other to the large animal area. The door to the large animal area opened, and a clinician came in leading a horse. He announced the signalment, i.e., that the horse was a 1½-year filly and that the "presenting complaint"¹⁸ was that the horse did not walk properly. He then asked the group what history questions they would like to ask. It is difficult to capture the speed and quiet intensity with which questions were asked and answered, but from all over the room questions from students (S) were fired at the clinician (C).

S: How long?

C: Since September the owners have noticed that it's not as agile as the others; it can't keep up; only wants to walk, not run.

S: Any evidence of trauma?

C: No.

S: How has the referring vet treated it?

C: No referring vet. This is the first time it had been seen.

S: How many limbs are affected? Is it a localized funny walk?

C: Not lame.

S: Off-balance?

C: Not coordinated.

S: What about its feed?

C: It eats well but has been competing less well for food.

S: Is it broken?

C: No. Only year and half old and never carried any weight. It was a large foal when it was born but as you can see now it's not really a large horse.¹⁹

The students never shouted their questions because of the presence of the animal. During the initial few minutes of intense questioning, the clinician appeared to answer the questions he could hear or the most persistent questions. In fact, he did not even look at the students. His attention was focused on controlling the horse. After he had total control of the situation and the questions slowed because most of the students were baffled by the case, he controlled the questioning by pointing to specific students. But, in fact, the overlapping, quick, intense questions were typical of questions students themselves were required to answer as they presented their cases in their clinical year. Students were constantly interrupted by faculty during their case reports.

Recognizing Problems

After the data from the signalment, history, and physical exam results have been compiled, they are then analyzed and used to create a hierarchical set of "problems." According to Weed (1969), this list of problems should not contain "diagnostic guesses" (p. 25). Rather the list "should simply state the problems at a level of refinement consistent with the physician's understanding, running the gamut from the precise diagnosis to the isolated, unexplained finding" (p. 25). These problems can represent a clustered set of symptoms, observations, or laboratory results. During their training, veterinary students are taught to recognize and name provisionally these configurations. Students, for example, through intense socialization experiences learn that only certain terms can be used to identify problems. A discussion with the record technicians revealed that new veterinarians would sometimes attempt to use the term *a poor doer* to describe an animal that essentially was not thriving. Such records would be returned for clarification because, according to the technicians, the term was too vague. It could describe too many conditions. The term *DIC* (death is certain), however, described a recognized syndrome—a constellation of signs that ensured that the patient was in an irreversible spiral toward death.

We can see this socialization process at work both in an exam question from a third-year clinical-studies course and from the evaluator's response to two answers. This type of test question was typical of exam questions in the final 2 years of the program.

Spot is a 6-year-old spayed female Dalmatian who is presented for a 1-year history of oozing sores mainly on the pressure points, i.e., elbows, stifles, and sternum. She is fully vaccinated and lives in town. Her appetite is fair and she drinks 2 cups of water per day. Lately, sores have appeared between her toes and she seems less inclined to go for her daily walk. The owner thinks these sores must be itchy because Spot chews at them and licks her abdomen. On physical examination we find that Spot is in good body condition and weighs 32 kg. The pressure point areas have multiple 3-4 mm nodules covered with reddish brown crusts. When the crust is removed, an excoriated area is revealed that oozes blood. The skin and hair coat on the ventral surfaces and flanks have a reddish yellow appearance. On closer examination, we see scattered papules and pustules on the abdomen. There is a slight ceruminous otitis but generalized seborrhea is not a prominent feature.

- a) Make a problem list at your highest level of understanding.
- b) Assess the problems individually or in compatible groups and give diagnostic plans for each.

Both clinicians and students in their final years at the college would recognize the required structure of the response, a structure derived from the medical record system. Students were trained rigorously to construct a problem list, to assess these problems (explain the interaction of signs and hypothesized problems), and then to suggest diagnostic or therapeutic plans.

There is no doubt, too, that students required both intensive background knowledge and mastery of this problem-solving heuristic in order to succeed on this type of question. The following are two examples of student responses to the first part of the question. In each case, the smaller handwriting is that of the student and the larger is the instructor's. The first example (see Figure 3) was successful; the second (see Figure 4) was not.

During a reader protocol, the instructor (Dr. P), a clinician deeply committed to teaching problem solving, explained her response.

Dr. P: So what the student is expected to do is formulate, read this, and put it into a list of problems so that they have to be able to interpret what this means and then interpret these problems at their highest level of understanding and this particular student [the writer of the response given in Figure 3] has done that with recognizing that oozing sores on the pressure points plus the papules and pustules on the abdomen are most likely bacterial in origin. And, as I said, bacterial dermatitis with oozing sores. So he's gone one step there to recognizing that these are

- a) Make a problem list at your highest level of understanding
- 3
- 1) Bacterial dermatitis with oozing sores.
 - 2) ^{hair loss} Reddish yellow skin ventrum and flanks
↳ Unusual pigmentation
 - 3) Ceruminous otitis
 - 4) Interdigital dermatitis
- Observe
↓ exercise

Figure 3: Successful Response

probably bacterial. He's not sure about the reddish yellow skin on the ventral and flanks. OK. So he's questioning that and putting it as a separate problem 'cause he doesn't think it goes with bacteria.

"Ceruminous otitis." Ceruminous is another word for a type of seborrhea or abnormal grease in the skin that involves just the ears. So he's able to recognize that may not necessarily go with this bacterial problem. Now he's also suggested that interdigital dermatitis is a separate problem and I would have expected maybe that he would have put that up here with the bacteria as it is bacteria as well. So there are three manifestations of a bacterial problem here—the pressure points where it is deep, the abdomen, and in between the toes. He also didn't pick up that 70 lbs is too heavy for a Dalmatian. And you could say that, well, maybe it isn't if she is overfed, but it says here that her appetite is fair. So she is not eating these people out of house and home. And he hasn't totally mentioned here that she doesn't want to go for her walks any more.

So this was worth 5 marks in each section to make a total of 15. So he got 3 marks, which wasn't bad.

If we compare it to this person [the writer of the response in Figure 4] who has listed one problem here that she calls "Dalmatian Bronzing Syndrome," which is maybe one reason why this dog has bacterial dermatitis but she has gone way ahead of herself and called that a problem, and we don't know that.

Researcher: She's gone right into diagnosis in fact.

Dr. P: Yeah. Well, a possible diagnosis. So she has made her problem list one diagnosis that may be correct and may not, so by restricting herself

- a) Make a problem list at your highest level of understanding

Problem List

- ① Dalmation Bronzing Syndrome Too presumptuous
- ② ceruminous otitis
 - Superficial Pyoderma
 - deep pyoderma
 - obese
 - ↓ exercise

Figure 4: Unsuccessful Response

in her problem list to a diagnosis which, in fact, is wrong, she has messed up the rest of the exam. She recognized "ceruminous otitis" but I've suggested here that her problem list should be a "superficial pyoderma, a deep pyoderma, obese and exercise," and that it is too presumptuous.

The clinician's differing evaluation of the two students' responses to part (a) of the test question clearly illustrates the socially constructed nature of "problems" and their embeddedness in the discourse of veterinary medicine. Only those deeply socialized into this speech community can construct the contingent categories recognized as problems or can linguistically encode the language of symptoms and signs into the specific language of problems.

The fundamental genre at work in this example, of course, is the exam question. But the relationship between the genre of examination and the genre of record keeping illustrates Bakhtin's point that a multitude of genres exist in any communicative context and that genres influence each other. In this case, the secondary, dominant genre is the exam and the primary genre is medical record keeping. One genre subsumes the other, but is also affected by the genre it subsumes. The linked discourse between records, student responses on exams, and faculty rejoinders to those responses suggests the dialogic and diachronic nature of genre.²⁰

Furthermore, records and the type of exam question they engender raise the question of addressivity. Addressivity, according to Bakhtin (1986) is a genre's "quality of being directed to someone" (p. 95). "Each

genre," he suggested, "in each area of speech communication has its own typical conception of the addressee, and this defines it as a genre" (p. 95). The addressee in the case of this exam, and I would argue in the case of medical records, is a reader who is prepared to "read" a great deal of tacit information into these accounts. Single words mean a great deal in these exams and records. Records are also organized in such a way that writers do not have to include many of the cohesive ties needed for readers outside the narrow circle of students and instructors at the college of veterinary medicine. Records are all organized in the same sequence. In composing progress notes, for example, students dap a case by collecting data, assessing data, and developing plans for treatment—ordering tests or specific drugs. These plans, of course, generate new data, and so the sequence continues. Students were penalized if they deviated from writing their progress notes in this order.

Bakhtin explains this type of addressivity as characteristic of "objectively neutral styles" (p. 98). Such styles "select language vehicles not only from the standpoint of their adequacy to the subject matter of speech, but also from the standpoint of the presumed apperceptive background of the addressee" (p. 98). In terms of the work of M.A.K. Halliday and R. Hasan (1986), much of the writing at the college was coherent (entirely comprehensible for readers at the college) without being cohesive (lacking the ties found in writing and speaking intended for diverse audiences). As Bakhtin suggested, such a style has consequences: "Objectively neutral styles presuppose something like an identity of the addressee and the speaker, a unity of their viewpoints, but this identity and unity are purchased at the price of almost complete forfeiture of expression" (p. 98). Of course, this "coherent" style was enormously useful for both the faculty and students. It allowed for rapid communication and efficient clinical problem solving. But at the same time this coherent style has consequences. As Dr. L discovered, some students expected that he also would be able or willing to read a great deal into a list of words.

Constructing Progress Notes

After an initial plan for the management of the case is drawn up, practitioners put their plans into action and then observe the results of their intervention through progress notes. These notes are central to Weed's contention that records could be audits of thought and

action. Instead of a haphazard collection of action, observations, and data, the POVMR notes are organized according to the problems identified in the initial assessment of the case. Under each problem, the practitioner lists data, assessment, and plans. As the initial intervention is put into action, symptoms and signs should change, and laboratory results will arrive. These results become data. As the data change, the practitioner's assessment or explanation of the disease or metabolic processes at work will also change. The practitioner might even reach a diagnosis. The assessment section records this interaction between data and the practitioner's knowledge of physiology and medicine. Finally, as the practitioner's understanding of a case increases, the plans change. These changes in plans, of course, produce additional data. Ideally, progress notes record this recursive interaction of data, assessment, and plans until the problem is resolved.

This model of problem solving embedded in the structure of the POVMR, in fact, influences many literacy events at the college. During their fourth year, students were expected to present their cases orally using the system. If they deviated from the system, faculty almost immediately reminded them to return to it. At the graduate level, students in the clinical area of the college could use this structure to present their complex cases to faculty in grand rounds. As noted earlier, undergraduate exams used the structure for testing purposes.

The POVMR is even influencing a genre of journal article written by clinicians. These texts are distinctly different from the experimental article.²¹ They generally begin with an abbreviated review of the literature; follow with signalment, history, and physical exam results; list problems and then record the clinicians' interaction with the case. The recursive action of data, assessment, and plans is evident in the following discussion involving a foal with "a grade III lameness." According to the authors, Hilary M. Clayton and Mark Hurtig (1987),

There was no improvement in the lameness following diagnostic local anaesthesia of either the left elbow or shoulder joints. Anthrocentesis of the left shoulder revealed hemarthrosis and decreased viscosity of the joint fluid. Samples of joint fluid were submitted for cytology and microbiology. (p. 707)

The first plan involved a local anesthetic. But this action produced no result (assessment). Therefore, the next plan involved anthrocentesis. This plan produced results: hemarthrosis and decreased viscosity of

the joint fluid (assessment). Consequently new plans were formulated—tests were ordered for the joint fluid.

Implications

The value of the POVMR as a genre should be evident. Veterinarians, like other health professionals, are becoming far more interested in large-scale epidemiological studies. Organized as it is around "problems," the POVMR offers the promise of extensive compilation of data. Veterinarians are able to access records using key words—the terms that describe problems—not only from their own hospitals but also from animal hospitals around the country. The system thus offers more research possibilities for clinicians, who, in fact, are now coming under increasing pressure from the college and the profession to publish research findings.

It is clear too that a recognizable problem-solving structure underlies the POVMR. According to both students and faculty at the college, the system is intended to model the way experienced practitioners solve complicated medical problems. Certainly, Weed (1969) wanted his system to function as a problem-solving heuristic, or as he said,

The medical record need not be simply a static, *pro forma* repository of medical observations and activity grouped in the meaningless order of sources . . . it can be problem-oriented, and thereby it can become a dynamic, structured, creative instrument for facilitating comprehensive and highly specialized medical care. . . . (p. vii)

Clinicians also reported that they enjoyed using the POVMR as part of their pedagogy, and judged by their degree of participation, students also appreciated the POVMR as part of a case approach in the classroom.

More important, perhaps, medical records were the site of intense coordinated social action at the college. Records recounted not only the phenomenological events of cases as interpreted and constructed by students and clinicians, but they were also used to justify medical decisions and to keep track of medical expenses. Furthermore, several students or staff members could add to and jointly construct the same record. The person creating the record was not as important as the organizational existence of the record itself. In fact, the central purpose of records is coordinated social action. Records not only record events but they also intervene in those events. When clinicians order

tests or treatments, they—through the records—are intervening in phenomenological events and creating new data—new language, new records.

As suggested earlier, however, a genre may have a complex and even contradictory relation to its social context. Research suggests that the decision to convert to the POVMR or POMR is not an easy one (Hudson & Farrow, 1981; Walker, Hurst, & Woody, 1973). The POVMR is more labor intensive than the SOR. Yet the clinical faculty at the college, especially the younger faculty, decided not only to convert to the POVMR but also to ensure that all their students used it, a decision that will have an impact on the profession throughout the region. In this area, virtually all veterinarians are graduates of this college. In addition, local practitioners send their difficult cases to the college. Records, of course, accompany these cases so clinicians are keenly aware of local record-keeping practices.

It is interesting to speculate about the clinicians' motives, motives probably derived from the present state of veterinary medicine. According to many spokespeople, veterinary medicine is in difficulty. These commentators point to lack of fit between present training procedures in North America and the needs of the profession. In their view, colleges are producing too many general practitioners trained in emergency medicine (Hopkins, 1987, p. 31), whereas the discipline needs more researchers in government and industry positions (Lewis, 1987, pp. 8-9), more specialists in species and organ systems (Lewis, 1987, introduction), and more consultants trained in herd health management (Guterbock, 1986, pp. 65-68). Veterinary colleges, unfortunately, are constrained in their attempts to respond to these demands both by the accreditation system, which still places an emphasis on producing general practitioners (Gage, 1987, p. 85), and by their own traditions—programs with heavy emphasis on memorization, despite the continuing assertion that veterinarians need to be good "problem solvers" (Kitchen, 1987, p. 92). The POVMR in many ways responds to these needs. In comparison with older systems, the POVMR is more conducive to research and to solving complicated medical problems, the kinds of problems specialists tend to see.

At the same time, veterinarians are experiencing other pressures. Issues of professionalism are being raised. Various professional associations are dealing with complaints related to professional standards, often derived from inadequate record keeping and/or communication difficulties between practitioners and their clients, and with complaints of a growing lack of collegiality between veterinarians,

perhaps due in part to the rise in competition for resources between practitioners (Grenn, 1988). The POVMR has been adopted by the college in partial response to these pressures—especially the demands placed on practitioners by insurance companies. I observed, for example, a class in which an insurance adjuster, an invited speaker, spoke at length to students about the necessity of good record keeping.

Yet acceptance of the POVMR is by no means unanimous. Several graduating students reported that the system was a great way to solve problems, but that they would not be using it in practice because it was too time-consuming and cumbersome. This view was confirmed by older practitioners, in contrast to clinicians, and also by the president of the local veterinary association. This individual has initiated a series of reforms with respect to medical record keeping, including peer review and more consistent descriptions of good record-keeping practices, but he feels the college's records are excessive.²² Another group within the college was also critical of the POVMR. Some research personnel, especially instructors in the Biomedical Sciences, believed that the system was too practice oriented, too focused on problem solving in individual cases. They felt that the POVMR actually made it more difficult for them to interest students in scientific research methods and issues related to basic scientific research.

A critical, dialectical analysis requires that researchers examine both the work that a genre performs and the work that it limits or prevents. As a researcher, I was called into the college to investigate the teaching and learning practices that constituted literacy, or the ways of speaking and writing characteristic of this group. Veterinarians believed that their students had a problem—the "green grad" syndrome, a staccato style of writing and speaking with both other veterinarians and clients. The more I investigated the actual reading and writing practices at the college, the more convinced I became that the record-keeping system itself was possibly implicated in this syndrome.

In fact, if Bakhtin is correct, it is entirely possible that these students were being exposed to too narrow a range of genres. Bakhtin (1986) suggested that "the better our command of genres, the more freely we employ them . . ." (p. 80). But the situation at the college and perhaps in other medical schools is such that characteristic ways of speaking and writing require only a few genres and not those that encourage a range of addressivity or fully transactional writing. These students might not be experiencing the full range of genres Rentz indicates that professional communicators need. Yet veterinarians need to communicate with a variety of clients and other professionals. In fact, one of

my central pieces of advice to the college was that they develop alternate genres to the POVMR, especially in some of their third-year courses, courses that the college was developing to encourage consultancy work. Consultancy work in veterinary medicine requires research skills because consultants assume responsibility for large groups of animals and need a good background in statistical analysis. Consultants must also be able to sell their services. They are in the business, in fact, of selling preventive medicine, rather than emergency services. Accordingly, instructors in these courses had the opportunity to develop oral and written assignments requiring research techniques and identifying specific audiences.

This exploration into the world of record keeping as seen through the analytic lens of genre defined as only stable-enough-for-now or a site of centripetal and centrifugal social and ideological action suggests that more detailed research is required on record-keeping practices. Many commentators (Cicourel, 1981; Donnelly, 1988) complain about the communication skills of medical professionals. At the college, I continually heard students told that they needed to distinguish between "symptoms" and "signs." This epistemological and ideological distinction, embedded in record-keeping practices, might make it difficult for practitioners to "hear" patients and then communicate with them. William Donnelly writing in the *Journal of the American Medical Association* suggested, for example, that physicians regularly construct patient histories, objective accounts, which exclude their patients' language. The limitations of this approach become apparent when the physician has to explain a diagnosis and its implications to a patient. They often lack a shared language. Finally, the intense question-and-answer sequence involved in teaching the POVMR (and perhaps the POMR) probably also has consequences. For example, students and clinicians interrupted clients during history taking. The Green Grad syndrome possibly has its roots in the discursive socializing practices of the college and one of its most important genres, its record-keeping system.

More important, as teachers of writing, particularly at the advanced level, we can teach our students to examine critically the genres that both constitute and are constituted by various organizations and disciplines. Genres have implications for the communicative abilities of a group. Genres appear to facilitate the development of some communication skills and minimize the development of other skills. Genres affect each other (witness the exam questions and the POVMR). Genres have complex histories and relations within dis-

course communities. Genres are evolving and function as ideological vehicles that represent the values of certain groups within the speech community and not others.

We need to teach our students to refuse simply to acquiesce to genres. As communicators, they need to be able to take them apart and see how they work and what they are actually doing or not doing within various communicative contexts.

NOTES

1. Of course, all participants in this study remain anonymous, as does the college itself. Participants told me that their challenges and problems were characteristic of veterinary colleges across North America. I wish to thank the college and the participants in this study for not only their kindness to me but also for their openness. They wanted a critical evaluation of their teaching and learning practices—a rare request.

2. Linda Brodkey (1987) has observed that “writing practices constitute literacy for a given community” (p. 7). I would also add that reading, listening, and speaking practices are deeply implicated in any descriptive definition of literacy.

3. In this article, I wish to adhere to the MLA practice of fully naming researchers when their work is first discussed. This decision acknowledges my agreement with feminist researchers such as Shulamit Reinharz (1992), who object to the “use of vague, impersonal, masculinist surnames” (p. 16). In fact, the use of impersonal surnames often hides the gender of researchers. This practice is no longer useful because, in fact, we need to recognize the contributions of female researchers within all fields.

4. For some, a classic ethnographic study requires that the researcher spend at least 6 months to 1 year in the “field.” But all research operates within constraints. The college permitted me to wander their halls and conduct any interviews I needed for a period of 6 months. I did, however, return the following semester with a series of reports and workshops explaining my findings. The response to these events also became part of my data.

5. Ten instructors from the first to the fourth year of the program selected three papers or exams representing a range of achievement from their class sets. During the taped interviews, they explained the differences between a first-rate and second- or third-rate response.

6. I cannot stress highly enough the value of conducting research into the beliefs and values of a professional community as expressed in their own educational and practice-oriented journals.

7. I used several strategies for interviewing faculty. First, I followed the hierarchy of the college by interviewing chairs of various departments. I asked them for the names of faculty interested in the issue of literacy, either positively or negatively. I followed the chairs’ suggestions but also balanced the data with interviews from faculty selected randomly from the college’s phone book. I was careful to interview both male and female faculty, and both older and newer faculty as well as faculty in all departments.

At all times, too, I was concerned about an appropriate balance. In other words, departments with fewer faculty did not receive as many interviews as larger departments.

8. These two activities—observation and analysis—exist, of course, on a continuum. But in ethnographic research, it is important to try to separate them.

9. My understanding of “speech community” and “discourse community” derives from Dell Hymes’s (1974) insight that a speech community is a necessary concept required to postulate the existence of a social rather than purely linguistic entity. At the college, there were a number of so-called discourse communities such as pathologists, clinicians, and biomedical scientists. Each group had its own interests (often competing) and specialized language. Yet all shared a common interest in the continuance of veterinary science and medicine. They all shared in the daily maintenance of the practices that constituted the college.

10. In her study, Pettinari observed the relationship between operating room reports and the surgical events that proceeded them. She noted that residents were not formally taught this genre and observed their consequent learning strategies. She also documented the loss of information that can occur as a result of the current system of creating these reports. Her study differs from mine in that it is more purely ethnomethodological. She concentrates on the social, cognitive, and linguistic practices that construct one type of document that goes into the medical file. My study is more ethnographic in that it concentrates on the larger-scale practices that constituted literacy and socialization at this college.

11. The categories in the *DSM-III* are similar to the phenomena of “problems” in the Problem Oriented Medical Record system. Problems are constellations of signs, symptoms, and observations figured into specific language. During their 5 years at the college, students were gradually introduced to this language and were required to use it, especially in medical records. However, as veterinary faculty made clear to me, problems are not necessarily diagnoses, as they seem to be in the *DSM-III*. The faculty did not want students to plunge into premature diagnosis. Problems seem to occupy a more provisional ontological status than the categories in the *DSM-III*.

12. In this study, both “emic” and “etic” were employed in accordance with their usage in anthropology. According to Marvin Harris (1980), an emic analysis “employs concepts and distinctions that are meaningful and appropriate to the participants” (p. 14). An etic analysis, on the other hand, “employs concepts and distinctions that are meaningful and appropriate to the observers” (p. 14).

13. It is hazardous to yoke dialectical thought and Bakhtin’s dialogic position. Bakhtin developed his perspective in opposition to dialectical reasoning. Yet both positions have much in common. Both work diachronicity and transformativity into their theories of social life and language. Both acknowledge the importance of complexity and contradiction. Bakhtin opposed the contradictory tendency of some dialectical thought to create systems such as the simplistic “thesis, antithesis, synthesis” solution.

14. For this reason, genre remains an emic phenomenon. To understand the most tacit aspects of a genre—especially when it is complete—researchers must consult its users.

15. This article explores only aspects of the recent history of medical record keeping, which has been evolving for centuries. For an interesting account of the formation of medical discourse, see Michel Foucault’s (1973) *The Birth of the Clinic: An Archaeology of Medical Perception*.

16. Of course, veterinarians collect “subjective” data—from the clients or owners. But this information has less value than if it were stated by patients themselves.

17. In human medicine, symptoms are what the patient tells the doctor.

18. The term *presenting complaint* is, in fact, an example of "vet speech." Animals arrived at the college "presenting" certain complaints—symptoms. In fact, all oral case reports began with the word "present" used either as a verb—as in "the animal was presented to the college with such and such a complaint"—or as a participle—as in "the presenting complaint was such and such."

19. The clinician was actually adding information that students should know. The signalment would have told more advanced students that the horse was probably too young to be broken. Demonstration rounds, however, were informal learning situations or initial socializing experiences for students. They got to try out their discipline's genres without penalty. Students enjoyed attending them.

20. The dialogic nature of genre is also illustrated by this article. This article has subsumed some of the genres of veterinary medicine and has been affected by those genres—especially in terms of language such as presenting complaint.

21. Several genres of journal articles are available to clinicians, including the experimental article. The experimental article, however, requires a control of variables—a radically different structure from genres associated with the POVMR. See Schryer (1992) for a more thorough exploration of the differences between these different and competing genres.

22. In preparation for the publication of this article, I asked college representatives to approve my use of their documents. At this point in the manuscript, a clinician noted in the margins that these practitioners had obviously never faced a lawsuit because if they had, they would have appreciated the POVMR.

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